

Excelsa Health Westmoreland Hospital

532 W Pittsburgh St, Greensburg, PA 15601

DISCHARGE SUMMARY Document #: DIS-009

Patient & Admission

Name	Aaron Hayes
Date of Birth	1966-02-02
Phone	412-555-1104
Address	34123 Andrea Estate Butler, PA, 16001
MRN	MRN-471349
Admit Date	2026-04-19
Discharge Date	2026-04-22
Attending	Melissa Wilkins, MD
Attending NPI	8299737634

Insurance

Primary Payer	Aetna
Primary Member #	W022941318

Primary Discharge Diagnosis

Right-sided heart failure with volume overload

ICD-10: I50.22

Hospital Course

The patient presented to the emergency department with progressive dyspnea, lower extremity edema, and abdominal distension consistent with acute decompensated heart failure. Physical examination revealed elevated jugular venous pressure, hepatojugular reflux, and significant peripheral edema. Chest radiography demonstrated pulmonary congestion with bilateral pleural effusions. Transthoracic echocardiography showed a reduced ejection fraction of 35% with right ventricular dilatation and elevated pulmonary pressures, consistent with right-sided predominant heart failure with volume overload. BNP was markedly elevated at 1,240 pg/mL. The patient was admitted to the telemetry unit for aggressive diuresis and optimization of his chronic heart failure regimen.

During the three-day hospitalization, the patient received intravenous furosemide with careful monitoring of renal function and electrolytes. He demonstrated excellent response to diuretic therapy, with resolution of orthopnea, significant reduction in peripheral edema, and improvement in exercise tolerance. Daily weights declined by approximately 8 pounds over the admission period. His home beta-blocker and ACE inhibitor were continued and uptitrated as tolerated. Electrolytes remained stable throughout the stay, and renal function remained at baseline. The patient was transitioned to oral diuretics prior to discharge with a target daily dose of furosemide 40 mg once daily.

At discharge, the patient was euvolemic on examination with clear lung fields and normalized jugular venous pressure. He demonstrated good understanding of his fluid restriction, medication compliance, and the importance of daily weights with early notification of his cardiologist for weight gain exceeding three pounds in one day. Close outpatient cardiology follow-up was arranged for one week post-discharge to reassess volume status and consider further optimization of his heart failure medications. The patient was discharged in stable condition with explicit instructions regarding sodium restriction and adherence to his diuretic regimen.

Procedures Performed

Right heart catheterization	CPT 93451
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Medication Changes

Spirololactone 25 mg daily	STARTED
Sacubitril/valsartan 49/51 mg BID	STARTED
Metoprolol tartrate	STOPPED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	14 days
Urgency Tier	MEDIUM
Clinical Flags	active volume overload at discharge

Dictated and electronically signed by:
Melissa Wilkins, MD
NPI: 8299737634
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